“Hacking Cancer 0.1, and the Endgame Strategy for Cancer” (Mark Taylor and Gabriele Gavazzi) [#156]

Brad Power
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“What we're starting to see – and I think it will only accelerate – is a bigger and bigger gap between what the standard of care is offering and what you can get with a doctor who's paying attention to the latest research.” – Mark Taylor

“What became key in our research is ultimately that the perfect oncology treatment is a combination of treatments. We explain some rationale to how you combine treatments.” – Mark Taylor

“My initial objective was to understand the effectiveness of a lot of the stuff that's not well researched. In general, I found very, very few things that worked effectively that were not studied in clinical research. That was my finding. So what we advise in the book is just be very wary of following people who are proposing things that don't have a clinical trial, ideally in humans, and if not in animals.” – Mark Taylor

“There are a lot of things in lifestyle that are very complicated, but some are very simple. Exercise is the most obvious one to start on. It works on so many different levels. We talk about different diets, but in general, I'm a bit suspicious of diets, because the studies aren't as grand as people expect them to be. I have a lot of patients who come to me who have shunned the standard of care and believe that a diet is going to fix them. The huge category of people like that because they've got distrust in the pharma world. Very, very few of them do well. There's a misperception of the benefits of diet. I'm not saying it cannot help, and some studies suggest they can help a little bit, but to the extent that is going to help is overestimated in the community.” – Mark Taylor

Meeting Summary

Cancer patients with a cancer diagnosis are confused about what to do alongside the standard of care to improve their outcomes. Many cancers have no good treatments and most treatments work for a while until they don't. There is a lack of trustworthy, unbiased sources of information on cancer treatments outside the standard of care, including on costs and effectiveness.

Mark Taylor and Gabriele Gavazzi are uniquely qualified to lead a discussion on what to do in this relatively uncharted territory. They have been studying outcomes of patients following “integrative oncology” (the combination of conventional cancer treatments with evidence-based complementary therapies) through four pillars:

1. Mark's own experimentation with integrative oncology solutions, such as cancer vaccines, anti-cancer IVs, mRNA gene silencers, cell therapies, hyperthermia, and Coley's toxins. He then moved into psycho-spiritual approaches, working with top healers such as Gabor Mate, Sholin monks, Qi Gong masters, and numerous shamans from Brazil and Peru.

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2. Tracking outcomes following patients in Mark's Facebook group of over 6000 patients. See https://www.facebook.com/share/g/194a6ekSgq/?mibextid=wwXIfr
3. Working with some of the top experimental oncologists in the world.
4. Research that keeps track of the research pool into integrative oncology in real time.

Mark and Gabriele have published a book called "Hacking Cancer 0.1", which includes all the clinical studies patients can use to improve their outcomes. It also includes a concept called the "endgame strategy", which is their opinion on the optimal way to treat stage 4 adenocarcinomas (one of the most common types of cancer occurring in the lungs, breast, colon, pancreas, and prostate) using the latest science.

What principles should guide your cancer treatment selections, especially for complementary treatments?

- Be evidence-driven and open to innovative approaches while maintaining scientific rigor
- Prioritize treatments with human clinical research evidence (not cell lines or animal models)
- Personalize your treatment selections based on your unique factors, like your financial resources, personal willingness to travel or take risks, and overall health and age
- Look for synergistic treatment combinations; consider low-dose combinations of treatments
- Use testing to focus and personalize treatments
- Focus on reducing side effects while maintaining treatment effectiveness
- Understand the specific biological mechanisms of potential treatments
- Be skeptical of marketing claims without solid scientific backing
- Continuously update and adapt your treatment approach based on the latest research

How can you integrate lifestyle changes, such as exercise and nutrition, into your treatment recommendations?

- Exercise is considered the most significant lifestyle intervention, working on multiple levels to potentially improve cancer outcomes.
- While you should consider your diet, you should be cautious about overstating its benefits, since diet studies aren't as conclusive as many believe.
- You should be careful about completely replacing standard care with lifestyle interventions.
- You need to consider personalized complementary treatments that are relevant for your cancer situation, rather than adopting a one-size-fits-all approach.

What are examples of personalized complementary treatments that are relevant for particular cancer situations?

- **pH modulation**: For pancreatic cancer patients, maintaining a urine pH of 7.5 and above was associated with significantly longer survival (29.9 months vs. 8 months for those with pH below 6.5).

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- **Blood sugar management**: For pancreatic cancer patients undergoing chemotherapy, managing blood glucose levels can potentially sensitize cancer cells to chemotherapy, with some studies showing remarkable responses.
- **Fasting**: Between chemotherapy treatments, fasting can help slow cancer progression and protect the body.
- **Dosage optimization**: Using lower, more targeted doses of chemotherapy, TKIs, and other treatments to reduce side effects while maintaining effectiveness.

How can you learn more about navigating integrative cancer treatment?

- Explore the Hacking Cancer Research Portal
- Join the Patient Led Oncology Facebook group
- Request a consultation with Mark Taylor (which includes access to the Hacking Cancer Research Portal)
- See previous discussions we have had on complementary cancer treatments, such as:
    - “Evaluating Complementary Therapies in Cancer Care” (Martin Lužbeták, MD, MS) [#108]
    - "A Patient’s View on Nutrition, Supplements, Integrative Oncology, and Complementary Therapies" (Robert Ellis and Glenn Sabin) [#33]
    - "Terrain and the Whole Person in Cancer Care” (Nasha Winters, ND, FABNO) [#95]
    - “Integrative Cancer Care” (Donald Abrams, MD) [#102]
    - “Cancer Scams: Don’t Get Taken” (Bapcha Murty) [#94]

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Meeting Notes

KEYWORDS

Cancer patient lab, integrative oncology, Mark Taylor, Gabriele Gavazzi, research portal, clinical research, patient-led oncology, cancer treatments, immunotherapy, metabolic treatments, AI tools, patient navigation, clinical trials, oncology research, patient coaching.

SPEAKERS

Mark Taylor (74%), Brad Power (22%), Gitte Pedersen (2%), Eric Dishman (1%), Len Sierra (1%)

CHAT CONTRIBUTORS

Allen Morris, Gitte Pedersen, Chad Magnussen, Helen, Raj Aji, Eric Dishman, Gabriele Gavazzi, Dennis

SUMMARY

Mark Taylor discussed the Patient Led Oncology research on integrative oncology, highlighting the Hacking Cancer Research Portal, which aggregates clinical research and provides tools for patients to navigate treatment options. Mark shared his personal experience with pancreatic cancer and the importance of evidence-based treatments. The portal, available in soft launch, offers a comprehensive database of treatments, clinical trials, and research articles, aiming to bridge the gap between standard care and experimental treatments. They emphasized the need for patients to be informed and proactive in their care, leveraging AI and personalized guidance to improve outcomes.

OUTLINE

Introductions

- Mark Taylor shared insights on integrative oncology.
- He shared his personal story of getting an early pancreatic cancer diagnosis and his research project on global clinics and treatments.
- Mark Taylor introduced the Hacking Cancer Research Portal, an online tool to help patients quickly access the latest research.
- The portal is patient-led, focusing on integrative oncology and following patients through experimental doctors.
- They aggregate clinical research.
- There is an increasing gap between standard of care and experimental treatments.
- The team includes Mark, Gabriele, and two research scientists, Dr. Jihan and Dr. Sahil, with a focus on keeping doctors up-to-date with the latest research.

Problems Addressed by the Hacking Cancer Research Portal

- Mark outlined the eight problems the portal aims to solve, including patient overwhelm, confusion about oncologist advice, and lack of transparency in integrative oncology.

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- The portal helps patients read clinical studies, judge good clinical trials, and access treatments and tests.
- The portal provides sources for research, service providers, and a framework for combining treatments.
- The portal aims to provide trusted information and a roadmap for patients and doctors, with a focus on end-stage adenocarcinomas.

Guiding Principles and Fundamentals of the Portal

- The guiding principles of the portal include catering to different patient resources and needs.
- The portal includes fundamentals like pH modulation, which showed significant outcomes in pancreatic cancer patients.
- The portal provides tools to help patients access healers and psychedelics.
- The portal includes treatment strategies, such as the Chow principle, which suggests reduced doses of multiple drugs can be as effective as full doses of a single drug.

Treatment Strategies and Immunotherapy

- A key concept is that core, synergistic treatments are combined with standard of care and with immunotherapy.
- The portal includes a circle of immunotherapy, breaking down the aspects needed for an immune response.
- The portal provides information on high mutational burden and the effectiveness of vaccines in clinical trials.
- The portal aims to help patients understand and navigate the complex world of integrative oncology treatments.

Practicalities and Future Plans

- The portal will be available for a soft launch, with a free tier and paid tiers for advanced patients and doctors.
- Mark plans to hold group coaching sessions and provide access to the portal for a month during coaching sessions.
- The portal will be updated in real-time, with new research added regularly.
- There is potential for a chatbot interface and a questionnaire to guide patients through the process.

Collaboration

- Brad Power suggested a potential collaboration with Cancer Patient Lab, including a chatbot interface and AI guides.
- Mark and Brad discussed the importance of lifestyle changes, such as exercise and nutrition, in cancer treatment.
- Mark explained the challenges of using AI tools like ChatGPT and the need for careful prompting to ensure accurate information.

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TRANSCRIPT

Brad Power
This is the Cancer Patient Lab. We're happy to have you all here today. We're honored to have Mark Taylor and Gabriele Gavazzi here to tell us about the latest insights they've been gathering. You may not know or may recall for some of you that Mark and Gabriele were on a session with us before. Mark has an incredible personal story of getting an early pancreatic cancer diagnosis and then pursuing a global search for clinics and treatments that he might consider. He turned that into a research project where he asked patients who went to clinics globally, what treatment they got and what the results they got were. Now he has one of the best databases I'm aware of, of all of these integrative oncology clinics that are in various places all around the world. So a very good resource for that. Gabriele is his right hand man, who, if you ask him a question about anything, you get a very long answer with 10 citations. Gabriele just does incredible research, and together, they put that kind of research that they've done into what is evidence-based, evidence-directed, best practices in care, which I'm sure they'll tell you more about. I rely on Mark and Gabriele to make sense out of the area of integrative oncology, which has some alternative approaches that are sometimes questionable, but seem to have some evidence. How do you make sense of it all? When I want to make sense of it all, I go to Mark and Gabriele, so just a personal endorsement before we get started.

We have a few housekeeping things we always say at the beginning. One is, this is for information purposes only. This is not medical advice. We try to bring information to our patients and caregivers that they can take to their medical team.

We are a 501(c)(3) nonprofit, and we depend on the kindness of donations to bring this webinar series to you, and would appreciate any donations you might make, which you can do on our website, where there's a Donate button, and it's very easy.

Mark Taylor 2:30
We are here to announce that it's been many years of research. It was an answer to the problem I had when I had a cancer scare with CTCs in that when you enter the alternative world, you don't know what to trust. I see it increasingly with patients.

It's an online portal to help a patient get up to speed quickly with the latest research. We've created tools to help with that. We're introducing what we're calling the Hacking Cancer research portal. And my organization is patient led oncology. We are a research organization. We research integrative oncology, so everything your oncologist is not giving you through four lenses.

I have my own experiences, and I could write a book about that. We also follow patients through our group and through patients I coach. Generally patients who are going to the top experimental doctors in the world come to me, and I get to find out what the experimental doctors are doing and reverse engineer their research. Increasingly, we're selling our research to doctors. So we're getting the circular information with those doctors, feeding information into them and understanding how their patients are doing. And then ultimately, we've become an

aggregator of clinical research. Very rarely do we see approaches working that have not been studied in clinical (phase I/II or mice) research. Most of the stuff we do these days is to read clinical papers. We have filters on all the research databases. We're looking at about 30 to 50 research articles a day. Part of my thought process behind creating this is that the problem between the standard of care oncologist and what's going to be available in the experimental space is going to increasingly get more sophisticated. Medical knowledge is doubling every three months, and still, the standard of care model takes 18 to – in some cases – 40 years to get a drug into the standard of care. What we're starting to see – and I think it will only accelerate – is a bigger and bigger gap between what the standard of care is offering and what you can get with a doctor who's paying attention to the latest research.

The team is myself and Gabriele at the core. We have two research scientists, Dr. Jihan and Dr. Sahil, who help us, and we have a team that also helps us from that group, which I'll introduce soon. We're going to introduce what is the Hacking Cancer Research Portal. It's intended for cancer researchers and doctors, because we realize doctors are too busy to keep a pace with the research, and the research is coming in so fast it's almost impossible for a doctor to spend their time researching. We can keep a doctor up to speed. We can be their right hand research partner, and then we're selling that service to any doctor who wants to come to us. The career creation of the portal, which will also turn into a book, is a collection of help from 20 or so patients and caregivers. Brad is mentioned here, thanks for your help on the architecture, and then Roger has made a contribution on the legal side. But it really is a collection of a huge amount of input that we've got on editing the book. A really big bit of help from Christine and Miki from Cancer Choices, and then Elenojr Liney, who did a huge amount of reading and checking all the clinical studies. So big thanks to them who spent a huge amount of time on it.

The intention behind Patient Led Oncology was also to form a patient collective, working together to shift cancer outcomes and to create transparency around what's happening in integrative space. It's a research database, but it's also a curation of the research. What we realize is that with the AI engines these days, getting access to the research is actually becoming increasingly simple for the patient. When we started this work, before the AI tools existed, it was very hard to actually find out where the research was. That's starting to improve now with the AI engines, but what's still difficult with the AI engines. You don't know what questions to ask unless you really know what you're doing. In some ways, what it can do is facilitate you using the AI engines. And actually, we talk a lot in the book about empowering you as a patient to use the tools, use these research databases to become your own navigator, because you're the only person really who can do this.

There were eight problems we're trying to solve. One is there's a common problem in patients. These days, you see them overwhelmed when they're diagnosed. They're told to follow a vegetarian diet, do fenbendazole, cannabis oil, and they're overwhelmed. The typical patient is bombarded with information, and there's too much to process. There's confusion about what their oncologist is telling them, when their oncologist is telling them nothing outside of standard of care works. We make that transparent exactly what's going on there with the understanding of phase three trials versus earlier stage trials, and patients just generally aren't aware that

these these studies exist, so it's a central database, and the intention is, over time, to capture all of the the human and animal studies that a patient could potentially benefit from in one place. I'm sure we haven't got it all. We tried to. What we want to do is use the network within the tool to help us build that database over time.

Patients also don't know how to read a clinical study, and in my opinion, there's a huge differentiator between a patient who can and a patient who can't. So we try and help them to learn how to read a clinical study with AI tools. Now that's become increasingly easy, and also how to judge a good clinical trial and a bad clinical trial. A lot of people on these online groups are referring to cell line studies and using that as a basis to do their treatment protocols. And in our opinion, that's a very weak way of doing it when there's a huge amount of human studies with very clear increases in outcomes or reductions in side effects. And what we try to do is steer patients towards the trials that actually have proven effectiveness in humans.

People need to be aware of how to access treatments and tests. So we provide sources, the research behind them, and service providers.

What became key in our research is ultimately that the perfect oncology treatment is a combination of treatments. We explain some rationale to how you combine treatments.

Finally, in general, in this world, there's a lack of trusted information. There's a lot of dubious information groups out there, and patients can very easily struggle and go down a wrong path into certain groups or reading certain books that maybe necessarily aren't the best one for them. The culmination of what we did was to try and summarize everything we could find in integrative oncology in a single page that a patient could put on their wall and use to navigate their journey. And this has been thousands of hours of thought put into this and thousands of dollars of graphic design work. And the intention was, could we represent everything in the oncology space in a single page? Now this can be used in many ways. First off, someone with the resources and the knowledge and the ability to absorb all this could build their protocol around this. We've focused the initial release on end stage adenocarcinomas. We've not covered neuroendocrine or blood cancers. We may do it if we have time in the line. We've narrowed it down just to adenocarcinomas for now. But in theory, all of these boxes could be useful in some ways for someone on that journey, and it's least worth someone who's on this journey going through a checklist to rule in or rule out one of these treatments knowingly.

The idea is to bring all the information to the fore in one go, and then for someone to understand it all and go. “Okay, this is what I can afford. This is what I can't afford. This is what I can't get. This is what I can get on a clinical trial.” It can also help someone understand, if they go to one of these experimental clinics, how advanced are they? You can take this to an experimental doctor and go, “What is this? What are these? What are you doing? What are you not doing?” And it really allows you to get a feel for the maturity and the specialism within certain clinics and doctors.

You can also use it to establish how likely clinical trials will work. You can ask the clinical trial coordinator, “Which mechanisms is this trial working on?”

Finally, it's basically a roadmap for our online portal and book. All of our chapters are organized like this. While this is a single page to navigate the book, beneath that are information sources that people can access, and within that, we've also tried to create a meta structure within it for finding useful informational sources, which I'll demonstrate. So breaking that down, I know it's a lot to absorb in one go, so the first thing is guiding principles.

What we realize, and what I particularly realize from coaching patients, is that every patient's different. Some have very minimal resources and cannot afford to go to some of the most sophisticated clinics in the world. So we cater for that. And then there are multi-millionaires out there who are willing to spend everything to keep themselves alive. It's possible that you're in your 80s and you're quite satisfied with dying and going through the hassle of traveling around the world to try and seek out a few extra years is maybe not worth the energy. So it's really just a series of questions to ask yourself and things to do to help you navigate this process. Not everyone is suited towards the same treatment protocol, and it's flexible, so we're always willing to take input.

I wanted to quickly demonstrate how that can be used now. Say, for example, a lot of people don't know how to investigate clinical trials. They don't know how to find them. They they're confused. So you can go into the online portal. All the information is shown on the left. The guiding principles are the start. So you can go to the guiding principle on investigate clinical trials. You can click on here how to find good clinical trials, and it will give you the clinicaltrials.gov website. And then what we'll also use it as a metadata for companies that help people find clinical trials. So we've listed one here, ill health. If you click on this, it provides some sophistication behind your mutations, your stage of diagnosis, to help you find clinical trials. And what we will do, over time there'll be more and more of these companies appearing, is we'll plonk these companies in there so the patient can at least access everything they need in this space. Second thing is fundamentals. When you do your research, there's certain things that are fundamental to any treatment journey that almost no matter what treatment you're doing, these things are almost definite to improve your outcomes. This is typically what's offered by naturopathic services. So when you see a naturopath, these are the type of things they follow, and it's quite complex. And for those that can afford it, it's quite useful having a naturopath on their journey to guide them through these things. And again, I'll delve into how we help patients through this. So one, one thing, pH modulation. Not a lot of people are aware of this. So you can go into the tool managing fundamentals, pH modulation. And then we all hold. Studies listed, including the science behind how pH modulation can improve outcomes. And this is an interesting one, because not a lot of people are aware of it. But pancreatic cancer, for patients who had a urine pH of 7.5 and above, they lived on average 29.9 months a late stage, versus eight months for those that had less than 6.5 so this is a very simple treatment that can be done at home, that can be measured with urine pH monitoring, sticks with potentially huge increases in outcome with some incredibly solid science behind it. So that's how the fundamentals work, and you could navigate each of the fundamentals through, through the portal.

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The next section is on innate healing. I went on a journey myself to spend a lot of time with therapists working in the mind, body space, and I've collated a framework of what I feel is useful in this area with the latest research, with tools to help you access healers if need be. So we can go through an example with this mastering section on innate healing. And let's say one common question I get is, Where can I access psychedelics? Given the latest research, there's some amazing research in psychedelics, helping with cancer, anxiety and depression. So you sort into trauma work. This gives some science behind trauma, some options, psychedelics, and then different psychedelics.

And then, if you're looking to find a retreat, you can click on here for resources. You can type in psilocybin, and then suddenly you've got a world map of all the places you can go to do psilocybin therapy. So what may have taken six months to a year for a patient to find they're there, pretty much instantly, if they're needing to look.

And then we come to treatment strategies. What we realized through our research is there are some fundamentals of cancer biology that are often not considered in standard of care oncology. One of your chats, Brad, was from Moffitt Cancer Center, talking about the Darwinian approach to cancer, so that's listed as an approach. And these are kind of high level thought structures around how you may deliver drugs and the science behind that.

So can go again into the book, and you can in the section on mastering treatment strategies. One of the most interesting ones that we came up against was something called a Chow Principle that's embedded. As a scientist who spent time researching different dosing levels, what he realized is that if he delivered a quarter dose of four drugs focused on the same target, he would get the same outcome of using a single drug at the full dose, but without the side effects. And I think we know from our observations and our research, from looking at thousands of studies, this is one of the biggest missing pieces in oncology today. There's no incentive at the moment for the oncology world to play around with different dosing combinations, and these are what the advanced clinics are doing quite often. Though they're reducing. We've seen chemo doses reduced from 100 to 15% and people still getting responses when they're combined with multiple other things.

Then we come to some of the science. We have a view, that can be challenged, that the right way to treat cancer for the best outcomes for longevity is immunotherapy, if done with a full understanding of how. The problem is, today in the standard of care, they've only got checkpoint inhibitors, which is one piece of a bigger jigsaw puzzle. We’ve created what we call a “circle of immunotherapy.” We have broken down the research of the things that need to be focused on to get an immune response. What we're seeing from following patients and following clinics is those that truly understand this can and are increasingly getting exceptional responses. It's really a question of finding the right doctors and clinics that do the right testing and give the right treatments to understand this all.

So one of the most common ones, I mean, most people know in the space that high mutational burden is what pretty much defines what works with standard of care medicine. However, increasingly now we have vaccines being delivered into the market, in clinical trials that can mimic to some degree, that process biologically. So again, going back into the book, so you can go into immunotherapy, we've broken it down. Step zero is knowing when to start or not. There are some situations where you may not want to go down this road. So we highlight that, and then a section on training the immune system. And what this is, is all the latest research on different types of vaccines immunogenic to an Excel that approaches increased outcomes from using them. A patient can use this to look at all the research and also find sources to get hold of these things moving on so and then we've come up with this concept of core, synergistic treatments. This has two lenses. One is that most of this is what you'll get with standard of care. We've looked into all the things that you can do to prove your outcomes with standard of care and reduce your side effects for each one. So if you go into the chemotherapy section, you'll find all the studies we found which boosted the effectiveness of chemo, and also all the studies we found which reduced the side effects. What we also realized in doing our research is that to the most part, all of these treatments can be done in synergy with immunotherapy if they're dosed correctly. The two exceptions really are chemo when its dose is too high, to the point when it becomes too inflammatory, and it destroys the immune system. And what the experimental clinics do is they reduce the dose or even make it metronomic, which has a lot of research to suggest it can work, and can work in synergy with immunotherapy, and then also with certain TK inhibitors. There's a lot of research suggesting that you can get almost as good results, in some cases, better results, with lower dose TK inhibitors. A lot of research suggests that when TK inhibitors are done in combination with immunotherapy, the synergies can be very powerful, and the rest should fairly obviously work with immunotherapy. And we give explanations of exactly how that can be done, and again, going into the book to see how that will work. Autologous cell transfers are a common one. People come to us. So you come here for cell therapies, different types of cell therapies, and you can find, say, research on dendritic cells, some advice to patients, some sources that we recommend.

And then testing. What we've also done is a tag on each treatment, what sort of testing you can use to focus the drugs more effectively. Throughout the diagram, we've highlighted what tests can be done to improve your outcomes.

In terms of the practicalities, the tool will be available for soft launch right now. I will be doing my coaching sessions. If people book a coaching session with me, they will be getting access to the website for a month, and I will be holding their hand for a month, including messaging and getting them to sort of use the book in tandem with myself through the coaching process, in order, really to test drive that the Pro that the book is working to try and iron out any flaws, any problems that patients are having. I'll also be starting to do group group coaching with, say, groups of 10, where we'll try and see if we can use the booking collection to help people along their way. And then the idea after that soft launch, which is started right now, is we'll offer it out to patients and doctors. There'll be a free tier that's free for some of the core information, and $19.99-a-month tier for a sort of advanced patient, patient researcher, and then a $99-a-month for doctors. We'll segregate out the stuff that's too complex for patients.

These are websites where you can get more information from Patient Led Oncology or Hackingcancer.health.

Brad Power 24:43
That's amazing. I'm so impressed to see all the progress that you and Gabriel have made. We've talked so let me just say the architecture, as I understand you refer to book, and that's sort of the body of knowledge that you and Gabriel have managed to gather. Yeah. And then you've got the portal, which is the access. Through the nice index that you've got on the side. We've talked about the idea of using a chatbot interface so that even even you I noticed was were sometimes struggling, like, where's the answer to that question? You know what the index is? And you know what all those things are? What if the patient were just able to voice the very request you were making, like, I want to understand about, you know, immunotherapy, something, something. Have you thought about that chatbot interface?

Mark Taylor 25:31
We're thinking about it. I want to be slow with it, because if the filtering is not correct, giving incorrect answers or something, we're kind of wary of. The thing with a chatbot is you're not really taking someone through a process. And we, kind of, within the book, people are encouraged to read the whole chapter. With the chatbot, you get a very small snippet. So we're trying to think about what's the best way of ensuring they're getting the necessary information also, but from a bite-sized bit of information, is something we're thinking through. And ultimately, what I'd like to do, subject to funding and finding the right people, is a tool that asks people a series of questions and really guides them on a journey. So a questionnaire as well as a chatbot guiding them through a process.

Brad Power 26:31
We should have another conversation about how we might collaborate with the Cancer Patient Lab. As you may know, in addition to having a chatbot that points at our webinar transcripts, like this one, it will be in there. We also have a journey map, which is what you were just talking about. We also have a project working on a guide to how people can use AI, how people can use a ChatGPT or Gemini or Claude. We started with defining some principles, which is just very similar to what you have for principles. There's a lot of possible collaborations that I'd like to explore with you.

I didn't notice lifestyle, like exercise and nutrition, too much in the big map you had. Is that in there?

Mark Taylor 27:32
It’s under foundations. We've got the fundamentals, exercise, sleep, optimal nutrition, and hydration.

Brad Power 27:46

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I see a question in the chat from Allen Morris: Is there a fee to join your Patient Led Oncology site?

Mark Taylor 27:59
No, there'll be a fee to join the hackingcancer.health one with all the information. We're in the soft pilot right now. If people want access, I want to hold people's hand through the process just to get feedback and stay close to people to make sure that it's working correctly. So if people book a consult on my Patient Led Oncology site, which includes a month's worth of access and a month's worth of coaching with me. The idea is to keep a very close relationship and to see if I can get patients using the tools by themselves as much as possible. The idea is at the end of September, when I feel I've got a handle that it's working, that we find out all the kinks, at the end of September, then we'll do a mass release.

Brad Power 28:47
Next question from Gitte Pederson: How do you ensure the data is updated?

Mark Taylor 28:54
We update it almost in real time. So literally, every week, we're updating the core book, and then we're trying to work out what the right uploading process is into the online version. So at the moment, we're updating probably twice a week on the online version. There might be a point where that comes once a month. That's a view. But we're kind of reading research. We probably come across somewhere in the region of five to 10 research articles a week that can be added. So there's a slight concern that it will just get too big, but we'll see.

Brad Power 29:32
One of your first statements in your presentation was the pace of new information. When we've talked in the past, the idea of a printed book just seems like, the moment you printed it, it would be out of date.

Mark Taylor 29:49
Yes, it's so true.

Brad Power 29:52
You have to have an online version. Now, I suppose you could have some of your principles and some other things and turn that into a book, because those might be timeless, but the specifics are changing continuously.

Mark Taylor 30:04
Certain people who would prefer a book to refer to. So we just have to think about exactly what we cut and what we include in that. There's some use in having a guidebook around. Not a lot of people want to be referring to the computer.

Len Sierra 30:25

“Hacking Cancer 0.1, and the Endgame Strategy for Cancer” (Mark Taylor and Gabriele Gavazzi) [#156]

Mark, you had you showed a map of the world, basically showing where one could get psilocybin treatment. Were those clinical trial sites or or outside of clinical trials?

Mark Taylor 30:41
They're outside of clinical trials. It's in countries where it's legalized. Basically it's increasingly being legalized all over the world. So in Spain, it's decriminalized. Portugal, Czech Republic, Germany now. So I think we're starting to see more and more places legalizing it.

Len Sierra 31:01
But I thought you did have some US sites.

Mark Taylor 31:06
There are a few states in the US where it’s legal.

Brad Power 31:11
okay, yeah, we had a session Len with from some folks at Johns Hopkins that have some clinical trials and offline or whatever. I can point you to the that, if it were, if you just go into our site and, you know, Site Search on psilocybin, I think though it'll probably Come up. Thank you. Anyone else with any questions? I

Brad Power 31:53
A question from Allen Morris in the chat: Have you engaged the academic Consortium for Integrative Medicine, founded by the academic medical centers? Are their consensus recommendations much more limited than yours?

Mark Taylor 32:06
We haven't engaged with them. I mean, I can have a look and seeing what the what they're suggesting. I mean, we're trying to be what we try to be in the in the book, and the portal is really kind of agnostic to institution and process, and it's just a research database. So we just let it's, it's, we laid out, we lay out all the research that people can benefit. It's quite possible that certain, certain certainly, as you get closer into standard of care, they have to be more careful, and they're not going to want to start using things in phase one and phase two studies. So we talk a lot about that in the book, in this risk, risk versus, you know, safety balance that people need to make. But I'll have a look see what, see what they say.

Brad Power 32:59
Mark from our previous conversations, I noticed Chad Magnuson is on it. He pointed out to me that some time ago, that when you're looking at integrative oncology, since it's such a wide field, as you laid out in that overall map, do you really need a trusted guide? And it's sort of how do you find a trusted guide? And as I've said, I would trust you. But how do you navigate amongst all of the many people I was going to use a pejorative, who are promoting alternative medicine options or complementary medicine options? How do you how do you think about because you've spent a lot of time looking at all those clinics, etc.

“Hacking Cancer 0.1, and the Endgame Strategy for Cancer” (Mark Taylor and Gabriele Gavazzi) [#156]

Mark Taylor 33:37
My initial objective was to understand the effectiveness of a lot of the stuff that's not well researched. In general, I found very, very few things that worked effectively that were not studied in clinical research. That was my finding. So what we advise in the book is just be very wary of following people who are proposing things that don't have a clinical trial, ideally in humans, and if not in animals. If it's a cell line study, you can kill cells in a lab with alcohol, with bleach, with anything, with water. One of the things we see people get distorted, is the easy way of marketing an approach is to look at this study, and the closer you get to the cell line, the more complex the study looks. Because you're looking at cellular biology. So it's very easy to look very sophisticated, very convincing in a marketing material. But if the key question to ask is, “Has it been studied in humans?” That's what we're trying to encourage with the book. There are all these studies in humans and and some in animals where we think it's useful to start there. We quoted the pH modulation thing that took outcomes from eight months to 29 months. That's a huge increase from a low cost approach. But the average patient doesn't know about that. They're going down off on a on a tangent, doing all sorts of stuff that's not well studied, and then, and really getting people to think about the marketing and how stuff is delivered to them, because the what the aim of the book is really to stop that distortion that people are having in mass hysteria and everyone's following an approach that everyone's following, even though there's no research.

We talk about survivorship bias. So fenben (fenbendazole) is a good example. There's not a single case report of someone clearing the cancer with fenben. And then you'll go, Well, sure, what about Joe Tippins? Well, he did it with immunotherapy. There's a few studies of people doing fenben with immunotherapy or chemo in combination. There's not a single one that has got a complete response with fenben alone. So people have this very distorted view of what works, because they're not asking the real strong questions to say, Where's the science? And that's, that's a core part of the early chapters of the book,

Brad Power 36:09
That's really fascinating, that you've come almost with an openness to all possibilities, and you've come back to almost the medical establishment perspective on what's safe and reliable?

Mark Taylor 36:20
Well, not no, because there's some subtlety there. The medical establishment will look for phase three studies. What we highlight is the phase one, phase two metadata studies that is unpatented and not going to phase three. That's where the bulk of stuff is, and it contains off label drugs, it contains supplements, it contains lifestyle changes, and increasingly, it contains combinations of modified doses of oncology drugs. And that's probably the biggest gap that we see in this world, in that there's no one filling that gap of trying 15% chemo with a 25% TKI with 10% dose combination of three checkpoint inhibitors. So, and that's where, when you really study the biology, the sophistication is on a long a non toxic, effective solution that can be matched through a test showing that they all will work.

Brad Power 37:17

I will follow up with you on this. Are you acquainted with CureMatch, which recommends drug combinations? I'll send you a connection. They're big proponents of drug combinations, and their whole business is as you have: combinations are better.

Mark Taylor 37:42
And it's the dosing that's important. It's crazy when you look at the studies. There are good studies suggesting a TKI with a chemo with a checkpoint inhibitor will work, but what they're doing in standard of care is they give 100% dose of each. The study has outstanding results, but they canceled it early because there's so many side effects.

Brad Power 38:00
Mina Nikanjam of UCSD talked about that. She works with Target Cancer on dosing for combinations. And just as you're saying, the method of action of many of these drugs are not the same. Everybody thinks it's like chemotherapy, so more is more toxic, but they're working on different methods of action. So you can take an immunotherapy and combine it with chemotherapy, and it's on a completely different wavelength. They're worried about combinations having combinatorial negative side effects. And in fact, because of the different methods of action, it's not as much of a concern.

Chad Magnusson asks, “Are lifestyle changes, specifically diet, specific for each type of cancer or one for all?”

Mark Taylor 38:44
The biggest benefit of lifestyle is exercise. There are a lot of things in lifestyle that are very complicated, but some are very simple. Exercise is the most obvious one to start on. It works on so many different levels. We talk about different diets, but in general, I'm a bit suspicious of diets, because the studies aren't as grand as people expect them to be. I have a lot of patients who come to me who have shunned the standard of care and believe that a diet is going to fix them. The huge category of people like that because they've got distrust in the pharma world. Very, very few of them do well. There's a misperception of the benefits of diet. I'm not saying it cannot help, and some studies suggest they can help a little bit, but to the extent that is going to help is overestimated in the community.

There's one of the most interesting studies in this space was with pancreatic cancer and chemo. Those with high blood sugar, with pancreatic cancer and for foreign ox had remarkably better results than those with blood low blood sugar because the chemo. Relied on fast dividing cells, and the fast dividing cells were feeding on the on the glucose. So, yeah, so, so we also talk about that in the metabolic treatments, it's you really need to pair up the metabolic analysis with the treatments you're doing and with the approach. And you also have to have a view on how effective that is, because sometimes it's a lot of energy. You know, if you tell someone to go on a low methionine diet, they've got to go out and work out where to buy all this food, which, which foods got this thing without included, and that's a huge amount of energy when there's some studies that that are like you go to the shop and double outcomes.

Brad Power 40:35
So that's a good segue to Jane McClelland Alan Morris asks who leads the one of the metabolic approaches? Do you follow Jane McClelland? And if so, how do your recommendations align or not align with hers?

Mark Taylor 40:51
So Jane talks on many topics, alright. So when you read her book, she's talking about cemetery, which has got some great results in studies. She's talking about vitamin C, and some which is starting to have good results in K rest cancers. So she talks about a lot of things, and I think her contribution to the oncology world is historically, going to be huge, because I think she popularized this world. And actually, I'm pretty sure I wouldn't be doing what I'm doing if it wasn't for her book. Her thinking is very good, but what happens with patients is they misinterpret her thinking because her book is complex. I don't think it's a starter book for patients. Any patient that picks up her book and tries to integrate a full metabolic pathway blocking is going to hit their head against the wall, getting all the drugs, realize there's no standard protocol, and realize some of the drugs have quite bad side effects, and you can spend a huge amount of energy doing something that, again, has not been studied in humans or animals in terms of the full approach. So it's not that I disagree with her book. She's incredibly smart, and she's got some great thinking behind it. And she caveats everything in terms of, she says, you know, do this metabolic approach, but combine it with a kill phase. No, she's kind of saying, it can slow the cancer down, but, yeah, don't stop doing the chemo, which is sent. But did you see some people misinterpreting her book? Because there's a lot of complexity there, and a lot of patients can't grasp at the start. I hope that's an answer to a complex a simple question, but a complex world.

Brad Power 42:24
I would support that from her presentation. That was one of my takeaways, was she might say that intravenous vitamin C is good in situation a Yeah, people would hear that as intermittent Vitamin C is good, yeah, yeah, across the board, or whatever. So it's, it's, that was the big takeaway for me, is it's, it's useful in a situation, and you have to get to that granularity of the situation to make it really effective. Yeah, yeah. Okay. From Helen Mark, thank you for this amazing presentation I'm done with frontline treatment for a very rare ovarian cancer for which there is not yet a protocol. So they used that of high grade serous ovarian cancer because that's the best that they had. But they clearly missed a couple of key mutations, like KRAS and BRAF. They also didn't address stem cells and so on. At this point, I've been sent away because everything is normal to them with an aggressive cancer that has a high rate for recurrence and metastasis. What do you tell people in my situation, I'm based in France, doing standard of care and also using integrative oncologist in London, prescribing off labels and supplements. How would patient led oncology help me in terms of direction at this point.

Mark Taylor 43:43
I mean, when the books out, you can you can read that, or you can book a consult with me, and I can give you access to the book for a month, and I can direct you to the research that could potentially help you so and then you could go through the guiding principles as a process. So

you know, unfortunately, and this is going to become an increasing problem, finances are going to become an issue with people. But that's not to say, if you don't have a huge amount of money, that there's not options. There is trans clarity on what what you can spend is very useful, and then, to some degree, more useful than actually spending money, because you can be very focused on what's best for me at that cost level. Because if it's a very open ended question, people come to me and say, What? What can I do? And said, Well, there's some treatments out there, but doctors that cards are 160,000 US dollars. So, so And there's, there's a huge amount. There's a huge amount, there's probably hundreds of different doctors you can go to, so, clarity on your situation, financially, willingness to travel, willingness to take risk in the console. I'll go through all this, and then I'll, I'll try and present options and some research that she can look into. Yeah.

Brad Power 45:00
Just a comment from Dennis, your pH comments are good. I have been using baking soda regularly for about eight months. Maybe it has helped me with my pancreatic adenocarcinoma.

Mark Taylor 45:14
Okay, yep.

Brad Power 45:19
From Allen Morris in the chat: please comment on the following ChatGPT reply to high glucose and pancreatic cancer. Some preclinical studies and cell lines or mice have suggested that high glucose levels may sensitize pancreatic cancer cells to specific chemotherapies like gemcitabine under certain conditions. However, these figures are not consistent across all studies. They're mostly not replicated in clinical human settings. Some of this effect may be related to metabolic vulnerabilities and cancer cells that could be exploited, but this remains largely experimental, and there's more, but you respond to that,

Mark Taylor 45:53
ChatGPT is a great example of one of our fears about whether the AI engines are going to take over our work and and the more we see, increasingly, I'm getting people sending me chat GPT or trying to inform me about things, but increasingly, what I'm realizing is the results are not that great, and you have to be very wary of using these tools, because a lot of people have the perception that they're getting the complete information. So I mean, I think what I'd ask people to do is combine it with a search on, say, one of the either on Google, and look for the study that's been talked about. And I can, I can bring it up, and there is a human study for this one in contradiction to what ChatGPT was saying.

Brad Power 46:57
The work we're doing on an AI project is exactly trying to figure out what is the confidence that the AI has and what it's recommending by sometimes, this is from Russ Hollyer, but playing different AIs against each other, or asking it to grade the confidence it has in the answer that it's giving.

Mark Taylor 47:21
Yeah, what I've noticed, though, is that you have to be very careful with the prompting you need to give it. Because what happens is it tends to filter it towards a conservative response with the engines, some engines are slightly different in that, in that they kind of shift you into and rightly so the phase three studies that have been done. So you have to be very careful when you're doing the prompting to really get to really know what you're really asking

Brad Power 47:58
Gitte Pederson asks in the chat: Have you used Open Evidence, which is a database that's being used by providers.

Mark Taylor 48:11
No, I'm not aware of that.

Brad Power 48:18
Okay, you should. It's been, it's like, recently had a funding round, and it's growing like 20% you know, a year. So it's growing crazy. So open evidence is worth checking. It's a kind of a chat GPT with it's pointing at reliable medical sources and its customers or providers.

From Eric Dishman in the chat: Based on your book, the knowledge you have brought together your interactions with doctors. Do you have a wish list for how to better train oncologists and the field, how would you train and prepare the workforce for all of this is headed?

Mark Taylor 48:48
Eric, yeah. No, it's with mine as well, because I spent a good chunk of my pension paying for these doctors in that I started realizing a lot of them weren't that well trained. And part of the idea behind this book is to train patients to a point where they're training their oncologist, and that they can demand more from their oncologist because they're educated. Even in the experimental space, there's not a lot of incentive for a lot of the experimental doctors to really keep on top of the research, because once the brand is created around a treatment protocol clinic, patients are coming no matter what. And what I what I want to do is educate patients and equip them with the research so that they can start saying, Well, why haven't you thought about that? Why haven't you thought about that? And I think if patients, I think, and it's going in that way, that groups like this, who are collectively educating patients, I think what will happen is there'll be a natural need for both the conventional world and the experimental doctors to up their game.

Eric Dishman 49:57
I mean, I'll, I just, I've helped about 3000 Patients. And overwhelmingly, still, even at the tier one cancer centers that we get help, get them access to, it's probably a 15 to 20% of the oncologist there can handle the complexity of the kinds of things that you're talking about. And obviously tools and things that are coming will help. But it's like, I mean, just, and that's the tier one places that you would think everybody would sort of be equally trained. It's great when they act as a team, right? But increasingly, the ability to do that is really hard. So yeah, I'm really, I'm really

frustrated by by how difficult it still is, even at the top places, to find people with the right knowledge and the way of thinking. So, yeah, it's got to be a book that could help.

Mark Taylor 50:44
Yeah, I mean, I'm not convinced that's going to change. Maybe the book will contribute a little bit, because, you know, I also understand the complexity of this world when you when you start trying to treat cancer based on the latest test, for example, in that you open the door for drugs, dozens of IVs that you know, it was so complex and messy, I'm not sure the conventional hospitals can manage it right now. And I think truly, when you understand the biology and read enough studies, what you end up with is like a Chinese medicine approach to oncology. You know that, you know small you know based on a test, okay, you need 10% of this chemo, you need 15% of this TKI, you need this off label drug. And how we as a medical world, come to that is a it's not easy to solve operationally.

Brad Power 51:48
Is people like a standard of care, because then they can follow something. It's a cookbook, it's prescribed, it's structured, yeah, straightforward and not personalized, necessarily, maybe population level, but at least I'm safe. I don't have to worry about liability. I can get worse. It's the whole system is steering everyone to not a personalized approach, but a more standardized approach. Yeah.

Mark Taylor 52:11
Solving that is difficult. How you had a great diagram in our first chat. It had chaos in one corner, and then the standard of care in the other. How you find that balance is complex. It's almost like if there was motivation to come up with the best oncology treatments, there'd almost be a new industry of people focused on technology gadgets to support this complexity. Because how do you give that right level of chemo and the right combinations in a safe way? It's not simple, right?

Brad Power 52:47
From Raj Aji in the chat: Does your book or portal address protocols for mitigating building of resistance to inhibitors and other treatments that are working?

Mark Taylor 53:00
Yes, yeah. We talk about, we bring in all this that we so each in the hacking section, which is all conventional areas, we've got a chapter on synergistic treatments and science of resistance. So we talk about how resistance forms, some of the mechanisms, and then we give the studies that are shown the ability to turn around resistance. So yeah, for all the studies we found, we've listed, I think that's an answer to one

Brad Power 53:32
From Allen Morris in the chat: For a pancreatic cancer patient undergoing chemotherapy. Are you recommending increasing blood glucose, and if so, by what route and dose?

“Hacking Cancer 0.1, and the Endgame Strategy for Cancer” (Mark Taylor and Gabriele Gavazzi) [#156]

Mark Taylor 53:41
I'm not recommending anything. I'm not a doctor, so I'm a research portal provider. But if you look for the study on it's a very interesting one, and I think it sparks controversy in this world, which is useful. Pancreatic cancer, high blood sugar sensitizes chemo. Now you can interpret that in any way you want. The results were outstanding, like really outstanding in the high blood pressure group with a number of Stage Four reaching complete responses and long term remissions. And that's incredibly rare. Now, what I'm happy to do is, what would I do? There is some benefits to fasting for protection of the body. And when you understand the biology of cancer, resistance seems to grow at a certain dose of substance. And if you go too fast against cancer, resistance forms quicker through DNA damage and inflammation, and that leading to resistant genes forming. So the most logical way is to try and protect the body as much you can at the start by fasting, doing as low doses of chemo and increasing it over time. And then when resistance forms, either increasing the levels of chemo or increasing blood sugar levels just before chemo. Yeah. So. So as resistance forms, you could see that the sugar as as something of making it more effective. So and then, but between the chemos, you'd want to fast, because it's been shown in studies to slow down the cancer between chemos. And that's mimicked by some of these experimental clinics in that they give insulin, which is a growth factor of a chemo. Just before chemo, incident, potentiated chemo is common in some general clinics.

Gitte Pedersen 55:29
Mark, I'm so fascinated and impressed by what you have done. We are trying to move outside of this standard of care because it doesn't work for most patients, but we're using a test to help navigate that. I would love to set up a separate call and see if there's, for instance, all the every patient will have. How do you improve effectiveness? And you just spoke to it of the treatment that they select. How do you prevent resistance? How do you optimize survival? And I, I just think what you have done is amazing.

Brad Power 56:30
Gitte has been doing research in RNA sequencing and interpretation of RNA sequencing, so she has some interesting tests that you might be interested in.

Mark, do you have any final words of wisdom or thoughts you'd want to leave us with in closing?

Mark Taylor 56:58
Thank you for the opportunity, and then just let people know that we'll be updating people on the launch of the book and the rollout through our Facebook group. Patient Led Oncology is the Facebook group. That's how I communicate to the patients or anyone interested. You can find me there. We're in the soft launch now. If people want a consult where we provide access to the book and coaching over a monthly period to help them on their journey, they can reach out again through my website.

“Hacking Cancer 0.1, and the Endgame Strategy for Cancer” (Mark Taylor and Gabriele Gavazzi) [#156]

CHAT CONVERSATION

00:33:28 Allen Morris: Is there a fee to join your patient led oncology site?
00:37:38 Gitte Pedersen: How do you ensure the data is updated ?
00:41:27 Allen Morris: Have you engaged the "Academic Consortium for Integrative Medicine" founded by Academic Medical Centers? And their consensus recommendations are much more limited than yours. ---
00:42:55 Allen Morris: Do you follow Jane McClelland and if so how do your recommendations align or not align with hers?
00:44:32 chad magnussen: Are the lifestyle changes, specifically diet, specific for each type of cancer or one for all?
00:44:54 DENNIS 1321 Smiling Fish: just a note. Your ph comments. Good! I have been using baking soda regularly for about 8 months. Maybe it has helped me. (pancreatic adenocarcinoma). Thanks.
00:45:53 Helen: Mark , thank you for this amazing presentation. I’m done with frontline treatment for a very rare ovarian cancer for which there is not yet a protocol. So they used that of High Grade Serous Ovarian Cancer bc that’s the ‘best that they had.’ But they clearly missed a couple of key mutations like KRAS and BRAF, they also didn’t address stem cells and so on. At this point, I’ve been sent away because everything is ‘normal’ to them. With an aggressive cancer that has a high rate for recurrence and metastasis what do you tell people in my situation? I’m based in France, doing standard of care. And also use an integrative oncologist in London prescribing off labels and supplements. How would Patient Led Oncology help me in terms of direction at this point?
00:52:11 Gitte Pedersen: Replying to "Mark , thank you for..."

Helen we validated in Ovarian cancer feel free to reach out glp@genomicexpression.com
00:53:57 Allen Morris: Please comment on the following ChapGT reply to high glucose and pancreatic cancer: Some preclinical studies (in cell lines or mice) have suggested that high glucose levels may sensitize pancreatic cancer cells to specific chemotherapies like gemcitabine under certain conditions. However:
These findings are not consistent across all studies.
They are mostly not replicated in clinical (human) settings.
Some of this effect may be related to metabolic vulnerabilities in cancer cells that could be exploited—but this remains largely experimental.
Example:
Some lab studies show that high glucose may increase expression of nucleoside transporters that allow more gemcitabine to enter the cell.
However, clinical significance remains uncertain, and high glucose still promotes overall tumor aggressiveness.
00:56:09 Gabriele G: We missed part of the previous question: diets need to be tailored to cancer type .Mark can comment on Ketogenic Diet for Glioblastoma
00:56:46 Helen: Replying to "Mark , thank you for..."

Hi Gitte, thank you. What shall I read before reaching out? Anything or just email you?

“Hacking Cancer 0.1, and the Endgame Strategy for Cancer” (Mark Taylor and Gabriele Gavazzi) [#156]

00:57:14 Gitte Pedersen: Have you tried OpenEvidense
00:57:50 Eric Dishman: Based on your book, the knowledge you have brought together, your interactions with doctors...do you have a wish list for how to better train oncologists and the field? How would you train/prepare the workforce for where all of this headed?
00:58:39 Gitte Pedersen: Replying to "Mark , thank you for..."

Just email me and we can take it from there
00:58:44 Helen: Replying to "Mark , thank you for..."

My cancer is mesonephric-like adenocarcinoma ovarian (MLA)
00:59:19 Gitte Pedersen: Replying to "Mark , thank you for..."

Just email me and we can take it from there - I will set up a call with you first ok
00:59:31 Helen: Replying to "Mark , thank you for..."

Thank you.
01:00:23 Raj Aji: Does your book or portal address protocols for mitigating building of resistance to inhibitors and other treatments that are working ?
01:01:54 Allen Morris: For a pancreatic cancer patient undergoing chemotherapy - are you recommending increasing blood glucose - and fi so by what route and dose?
01:02:18 Helen: Replying to "Have you tried OpenE..."

Thanks for this, checking it out on my own cancer and will compare studies I’ve already accumulated